QA LEADERSHIP ACADEMY

QA Current-State Assessment Template

A structured way to capture where quality stands today — evidence, not opinion — so your strategy is built on the real starting point.

Purpose

Before you can decide where quality should go, you need an honest, evidenced picture of where it is. This template structures that picture across the dimensions that matter — people, process, tooling, environments, metrics and culture — and pins each observation to evidence and to the risk it creates. It is the bridge between diagnosis and strategy: everything in your eventual strategy should trace back to a finding here.

When to use it

- At the end of your onboarding period, to consolidate what you learned into one shareable document.
- As the input to a QA strategy — your strategy should answer the problems this assessment surfaces.
- When making the case for investment, so the "before" picture is documented and defensible.
- As a baseline you can re-run later to demonstrate progress.

Instructions

For each dimension, record three things: the current state (what is actually happening), the evidence (how you know — an observation, a number, a document, several consistent interviews), and the resulting risk or impact. Discipline on the evidence column is what separates a credible assessment from a list of opinions. Keep it factual and neutral; save recommendations for the strategy. Where you genuinely do not know, write "unknown" rather than guessing — a named unknown is itself a finding.

Worked example

A current-state assessment for Northstar Digital, drawn from the onboarding diagnosis.

Dimension	Current state	Evidence	Risk / impact
Team & skills	6 QA for ~35 developers; strong exploratory testers, one automation engineer, one recent hire ramping. No performance, security or accessibility specialist.	Team structure; skills heatmap; one-to-ones.	Whole capability areas (security, performance) uncovered; over-reliance on individuals.

Dimension	Current state	Evidence	Risk / impact
Engagement model	QA is centralised and pulled into squads late, once stories are code-complete.	Observed two releases; consistent across squad interviews.	Defects found late and expensively; QA blamed for slow releases it was too late to prevent.
Test approach	Broad manual testing with a ~5-day regression pass before major releases; not risk-targeted.	Release observation; regression tracker.	Slow releases; effort not concentrated where risk is highest.
Automation	~1,800 UI Selenium tests, ~6-hour run, ~25% flaky, QA-owned; developers do not touch it.	Suite dashboard; interviews with Dan and developers.	Low trust in results; maintenance burden on one engineer; slows rather than speeds releases.
CI/CD	CI builds and runs unit tests on PR; deployment semi-automated with human approval to production.	Pipeline configuration; deployment walkthrough.	Quality signals not gating releases; safety depends on manual judgement.
Environments & test data	Shared staging unstable and frequently holding "someone else’s data"; no test-data strategy.	Direct observation; repeated across interviews.	Unreliable test results; wasted investigation time; blocks dependable automation.
Requirements	Inconsistent — some squads write clear acceptance criteria, others hand over a Figma link and a sentence.	Sample of recent stories across squads.	Ambiguous scope; defects rooted in misunderstanding rather than code.
Defect management	Jira workflow exists but severity/priority applied loosely; production defects trending up.	Jira export; defect trend over recent months.	No reliable prioritisation; escaping defects not systematically understood.
Metrics	Reports test-case counts and bugs found; little on escaped defects, change failure rate or customer impact.	Current QA report pack.	Cannot answer the questions the board actually cares about; QA value invisible to execs.
Culture & relationships	QA seen as a downstream gate; one squad lead unconvinced of QA’s value; team wary after a failed automation push.	Stakeholder interviews (Product, Payments); team one-to-ones.	Change fatigue; resistance to a new "automate everything" message; trust must be rebuilt.

Blank reusable version

Dimension	Current state	Evidence	Risk / impact
Team & skills			
Engagement model			
Test approach			
Automation			
CI/CD			
Environments & test data			
Requirements			

Dimension	Current state	Evidence	Risk / impact
Defect management			
Metrics			
Culture & relationships			

Common mistakes

COMMON MISTAKES

Writing opinions in the evidence column ("everyone knows staging is bad") instead of a verifiable observation or number. Slipping recommendations into the assessment — the fix belongs in the strategy, and mixing them makes the diagnosis feel like an agenda. Assessing only the process and ignoring culture and relationships, which are usually where change actually succeeds or fails. Overstating confidence on things you have not verified. And burying the whole thing in a thirty-page report nobody reads — one page per dimension, evidenced, beats a treatise.

How to interpret the results

- Cluster the risks. Several findings usually point at one root cause — at Northstar, late QA engagement drives the slow regression, the late defects and the blame. Fix root causes, not symptoms.
- Separate foundational problems (environments, test data) from downstream ones (automation trust). Foundations first — everything downstream depends on them.
- Note where the evidence is weak or missing. Those are the areas to investigate further before you commit strategy to them.
- Watch for a mismatch between what people say and what the data shows — that gap often points to the most important cultural finding.
- If nothing here creates meaningful risk, either you are in unusually good shape or you have not looked hard enough at the defect and incident data. Usually the latter.

INSIDE STLC PRO TIP

Keep this assessment factual and neutral, then hand it to your stakeholders before you propose anything. If they read it and say "yes, that is us", you have earned the right to propose a strategy. If it provokes an argument, better to have that argument about the diagnosis now than about the cure later.